

إسم المريض رباعياً : فاطمه عبدالرحمن لاشين ~
المحافظة : حلوان الحي / المدينة : حلوان
رقم البطاقة : 25910052200564 رقم التليفون : 01226263147
العمر(تاريخ الميلاد) : 05-10-1959 المهنة : لاتعمل
العنوان : مساكن صلاح سالم
تاريخ تسجيل الإستمارة : 24-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

**رئيس
المجلس الطبي المتخصص**

--

Name: فاطمه عبدالرحمن لاشين.~~ National ID: 25910052200564	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني
مستشار

٩ - ٩



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
مجلس الوزراء

09/09/2026

MDT FORM

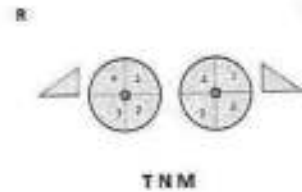
Name:	Fatma Abd Elrahman Lashen	MRN:	2936
Age:	67 Y	ID:	25910052200564
Menopausal Sta	Post	Phone:*	01100322395
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:	DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C trez	newly developed bony lesions	
Surgical History:*	TAH-BSO> benign , Cholecystectomy , Rt MRM 2020	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX: IDC G2
ER: 7/8 **PR:** 7/8 **HER2:** 0 **KI67:** 10%

History:

A case of Right breast cancer since 2020, cT2 N2 luminal A , received neoadjuvant AI (did not receive chemo because of her cardiac condition)> Underwent Rt MRM 10/2020 multicentric invasive duct carcinoma grade 2 ypT3 N1 (3/28) IHC on surgical pathology ER 6/8 PR 2/8 Her 2 0 Ki 67 7%. Received PORT and on adjuvant Arimidex / Zometa since 2020. C/O of low back pain of 3 months duration, did CT Chest and Abdomen +C 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan. O/E tenderness at dorsal spine , bilateral hips and left shoulder, no neurological affection



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Metastatic bone , query lung

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

CT Chest and Abdomen (no pelvis) with contrast 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lun base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
 No previous CTs done.

Recommendations:

MDT Outcome & Plan

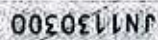
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SIGNATURE

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البطاقة سارية حتى ٢٠٢٩/٠٨/٣١



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ॐ नमो भगवते वासुदेवाय

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بسم الله الرحمن الرحيم
الحمد لله الذي هدانا لهذا الذي كنا لنهتدي لولا أن هدانا الله



POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -
Known case of breast cancer.

1. Chest:

- Right breast mass is seen measuring about 4.2 x 3.5 cm with two enlarged right axillary lymph nodes largest measuring about 2 x 1.3 cm.
- A single middle lobe pulmonary micronodule is seen measuring 5 mm.
- No other sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Right hilar calcified lymph node is seen.
- Normal cardiac size.
- No pleural or pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.

OPINION:

- Right breast mass and pathologically enlarged right axillary lymph nodes.
- Single right pulmonary micro-nodule for follow up.
- Fatty hepatomegaly with no focal lesions.

Patient's Name: Fatima Abdulrahman Lashin Hassanain
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Multi-Slice Biphasic Post Contrast CT scan of the Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

- **Bilateral few small pulmonary nodules** seen at both lung parenchyma mainly at lower lobes the largest noted at left lung base measures about (5.5mm) ... Likely metastatic
- **Bilateral basal atelectatic bands.**
- **Patent trachea,** main and segmental bronchi.
- **Normal both hilar region.** No hilar masses or lymphadenopathy.
- **Clear pleural surfaces** with no evidence of pleural effusion.
- **Normal cardiac size,** no pericardial effusion
- **Asymmetrically enlarged left thyroid lobe,** calcified nodule at right lobe **For thyroid work up.**
- **Mild enlarged liver.** No abnormal attenuation values or enhancing lesions.
- **No dilated intra-hepatic biliary radicles.**
- **Normal caliber patent Portal Vein**
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- **Normal size, and contrast excretion by both kidneys.** Normal parenchymal thickness. No renal, masses stones or back pressure changes.
- **Normal both supra-renal glands,** no masses.
- **No ascites**
- **Normal aorta, I.V.C.**
- **Normal filling of the urinary bladder** with smooth inner surface and uniform wall thickness.
- **Destructive sclerotic bony lesions** seen involving the right scapula (glenoid, acromion), Sternum, D11 vertebral body, Another ill defined bony lesions seen scattered at few ribs bilaterally with cortical disruption....Metastatic for bone scan.





Date : 03/09/2024	Patient Name: Fatma Abd Al Rahman Lasheen
Age : 65 Year(s)	Exam : X-RAY CHEST
Pat. ID: 11519	Medical ID: 631/2021

Plain X-ray Chest (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Border line cardiac size
- No bony chest lesion.

*Best Regards,
Dr Dina Gaber, FEBR,*

Not Approved

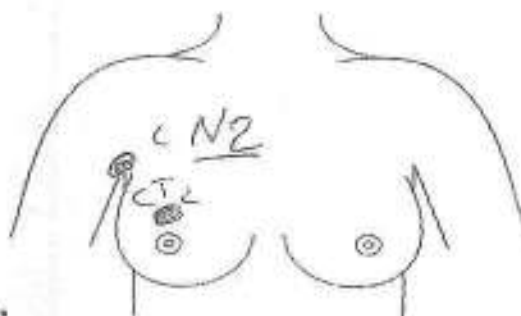


Name: فاطمة عبد الرحمن الشبرا
Age: 50 Y
No: 50 Y

23/1/2020

MDT FORM

Clinical data:



Reason of discussion:

MDT Outcome and Plan:

For metastatic work up (Not done) bone & CT
CT & N2 tumor then MDT
Neoadjuvant chemotherapy after clinical effect

Signature:

Dr. F. Al-Shera

Dr. F. Al-Shera
Dr. F. Al-Shera
Dr. F. Al-Shera

IBC
Luminal A
HIN, DM4, ID
fast cov
/ CT scan



Date : 03/09/2024	Patient Name: Fatma Abd Al Rahman Lasheen
Age : 65 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 11519	Medical ID: 631/2020

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- Enlarged in size (about 17 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- IS SURGICALLY REMOVED
- CBD is not dilated.

Spleen: -

- Average size (About 11 cm), displaying homogenous echopattern.

Kidneys: -

Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.

-Para-aortic and pancreatic area: no gross lesion

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -: is surgically removed

No free fluid in the Douglas pouch.

Opinion:

Bright hepatomegaly.

Best Regards,
Dr Dina Gaber , FEBR

Not Approved



Patient name: Fatma Abdel Rahman Lasheen

Date: 23-5-2022

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Technique

- left MLO and CC views
- left 3D MLO DBT (Digital Breast tomosynthesis)
- MRM & breast ultrasound

Report:

- Right MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Left Breast:
 - ACR B
 - No suspicious micro calcifications or architectural distortion detected.
 - Scattered benign calcifications.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post right MRM status showing no evidence of recurrence (BIRADS 2)
- left breast: benign calcifications (BIRADS 2)

Best regards,
Dr. Peter Fahmy



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m7857

Female :	النوع	فاطمة عبدالرحمن لاشين	اسم المريض
القاهرة :	العنوان	60 Y	السن
22/12/2019 12:11 :	تاريخ استلام العينة		تاريخ اخذ العينة
25/12/2019 13:42 :	تاريخ التقرير		طالب التحليل
		صحة المرأة	عناية
		صحة المرأة	

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody - CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.
Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.
Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of > 60 % of tumour cells (core biopsy)
PR: strong nuclear staining of 60 % of tumour cells (core biopsy)
HER2/neu protein over expression : NO membrane staining of tumor cells (core biopsy) .
Ki67 : positive nuclear staining of about 10 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 7 / 8)
PR: POSITIVE (SCORE 7 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE
Ki67: THE PROLIFERATITON INDEX OF THE TUMOUR IS ABOUT 10 % .

Pathologist:

د/تريزا ماهر

Lab Director: د/ تريزا ماهر



19 El-sheikh Rehan Street, Abeideen, Cairo
Tel: (165)7948544 - 7950096 Fax: (202)7948544
Mobile: 01144297292
Website: http://www.healthlabs.net
Email: pathologychl@yahoo.com
Ebook: قسم الباثولوجي - الإدارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها



اسم المريض : فاطمة عبد الرحمن لاشين

تاريخ الخدمة: 5/10/2020

Immunohistochemistry report

Lab-No.:20/608

-Procedure:

-Sections immune stained for ER, PR, Ki67 and Her2neu using supersensitive monoclonal antibodies with Roche envision (Dextran substrate polymer) flex detection system. Pretreatment for heat induced epitope retrieval (HIER) was performed using flex target retrieval solution. Blocking of endogenous peroxidase was performed using Roche peroxidase blocking reagent DAB was used as a chromate.

Results:

ER: (6/8).

PR: (2/8).

Her2: Negative, score = 0

Ki67: 7%

N.B: the slides are delivered to the patient.

Prof.Dr. Hussain Elsayed

Date: 29-Sep-2020 Time: 11:55 am

SIGNATURE

For CDK 4/6 + Fasiodex
Review Bone Scan and Palliative RT to the indicated areas if needed.
Zometa / 3m after dental assessment

MDT Outcome & Plan

Recommendations:

CT Chest and Abdomen (no pelvis) with contrast 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm. Likely metastatic - Destructive sclerotic bony lesions at right scapula, Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan.
No previous CTS done.

Mets Workup:

Biomarker/Molecular

Sonamamography:

Pathological Staging:

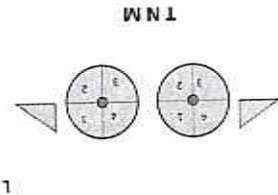
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History:

ER: 7/8 PR: 7/8 HER2: 0 KI67: 10%
IDC G2

BX:



Clinical Data:

Name: Fatma Abd Elrahman Lashen		MRN: 2936
Age: 67 Y		ID: 25910052200564
Menopausal Sta Post		Phone: 01100322395
NRS SCORE:		Marital Status:
BMI:		Family History:
Gender		Reason of MDT:
CO Morbidity: DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C trea		ECOG Performance Status
Surgical History: TAH-BSO> benign, Cholecystectomy, Rt MRM 2020		0 00 1 20 30 4

MDT FORM

09/09/2026



وزارة الصحة
مملكة العربية السعودية
لحجج صحت المرأة المصرية



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2591

وزارة الصحة
مملكة العربية السعودية

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OPINION:

- **Right breast mass and pathologically enlarged right axillary lymph nodes.**
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Patient's Name: Fatima Abdulrahman Lashin Hassanain
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Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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09/09/2026

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Recommendations:

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BMI:		Family History:	
Gender		Reason of MDT:	newly developed bony lesions
CO Morbidity:	DM, HTN, Cardiac ISHD 3 stents last 2019, Hepatitis C tre	ECOG Performance Status	☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
Surgical History:*	TAH-BSO> benign , Cholecystectomy , Rt MRM 2020		

Clinical Data:

R

L

BX: IDC G2
ER: 7/8 **PR:** 7/8 **HER2:** 0 **KI67:** 10%



T N M

History:

A case of Right breast cancer since 2020, CT2 N2 luminal A , received neoadjuvant AI (did not receive chemo because of her cardiac condition)> Underwent Rt MRM 10/2020 multicentric invasive duct carcinoma grade 2 ypT3 N1 (3/28) IHC on surgical pathology ER 6/8 PR 2/8 Her 2 0 Ki 67 7%. Received PORT and on adjuvant Arimidex / Zometa since 2020. C/O of low back pain of 3 months duration, did CT Chest and Abdomen +C 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lung base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan. O/E tenderness at dorsal spine , bilateral hips and left shoulder, no neurological affection

Clinical Staging:

Metastatic bone , query lung

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Met Workup:

CT Chest and Abdomen (no pelvis) with contrast 8/2026> Bilateral few small pulmonary nodules seen at both lung parenchyma at lower lobes largest at left lun base measures 5.5 mm likely metastatic - Destructive sclerotic bony lesions at right scapula , Sternum, D11 vertebral body, other ill defined bony lesions scattered at few ribs bilateral with cortical destruction metastatic for bone scan. No previous CTs done.

Recommendations:

MDT Outcome & Plan

For CDK 4/6 + Fasiodex
Review Bone Scan and Palliative RTH to the indicated areas if needed.
Zometa /3m after dental assessment

SIGNATURE

about:blank

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:-

Known case of breast cancer.

1. Chest:

- Right breast mass is seen measuring about 4.2 x 3.5 cm with two enlarged right axillary lymph nodes largest measuring about 2 x 1.3 cm.
- A single middle lobe pulmonary micronodule is seen measuring 5 mm.
- No other sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Right hilar calcified lymph node is seen.
- Normal cardiac size.
- No pleural or pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.

OPINION:

- Right breast mass and pathologically enlarged right axillary lymph nodes.
- Single right pulmonary micro-nodule for follow up.
- Fatty hepatomegaly with no focal lesions.

Patient's Name: Fatima Abdulrahman Lashin Hassanain
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Multi-Slice Biphasic Post Contrast CT scan of the Chest and Abdomen with Coronal and 3D Reconstructions Revealed:

- Bilateral few small pulmonary nodules seen at both lung parenchyma mainly at lower lobes the largest noted at left lung base measures about (5.5mm) ... Likely metastatic
- Bilateral basal atelectatic bands.
- Patent trachea, main and segmental bronchi.
- Normal both hilar region. No hilar masses or lymphadenopathy.
- Clear pleural surfaces with no evidence of pleural effusion.
- Normal cardiac size, no pericardial effusion
- Asymmetrically enlarged left thyroid lobe, calcified nodule at right lobe For thyroid work up.
- Mild enlarged liver. No abnormal attenuation values or enhancing lesions.
- No dilated intra-hepatic biliary radicles.
- Normal caliber patent Portal Vein
- Normal size, shape and parenchymal density of the pancreas. No enhancing lesions.
- Normal size, shape and density of the spleen. No focal lesions.
- Normal size, and contrast excretion by both kidneys. Normal parenchymal thickness. No renal, masses stones or back pressure changes.
- Normal both supra-renal glands, no masses.
- No ascites
- Normal aorta, I.V.C.
- Normal filling of the urinary bladder with smooth inner surface and uniform wall thickness.
- Destructive sclerotic bony lesions seen involving the right scapula (glenoid, acromion), Sternum, D11 vertebral body, Another ill defined bony lesions seen scattered at few ribs bilaterally with cortical disruption....Metastatic for bone scan.

